

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

August 13, 2025

MEMORANDUM FOR: (b)(7)(F),(b)(6) (b)(7)(C), DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(7)(F),(b)(6) (b)(7)(C)

THROUGH: CAPT (b)(7)(F),(b)(6) (b)(7)(C), MD, MPH, CCHP, FAAFP
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Chief, Investigations Unit
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FROM: LCDR (b)(7)(F),(b)(6) (b)(7)(C), RN
Investigator, Investigations Unit
ICE Health Service Corps

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SUBJECT: Mortality Review – Report of Findings
Marie Ange BLAISE, (b)(6),(b)(7)(C)

Executive Summary:

On April 25, 2025, the U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of alien Marie Ange BLAISE, (b)(6),(b)(7)(C). ICE detained Ms. BLAISE, a 44-year-old Haitian female, at Broward Transitional Center (BTC), in Pompano Beach, Florida (FL), from February 20, 2025 to April 25, 2025, the date of her death. Ms. BLAISE's autopsy and death certificate are pending.

Prior to her detention, on February 12, 2025, the U.S. Customs and Border Protection (CBP) encountered Ms. BLAISE at Henry E. Rohlsen International Airport, in Saint Croix, U.S. Virgin Islands, while attempting to board a flight to Charlotte, North Carolina.

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On February 13, 2025, CBP charged Ms. BLAISE with removability pursuant to Section 212(a)(9)(A)(ii) of the Immigration and Nationality Act. On February 14, 2025, ICE Enforcement and Removal Operations (ERO) Miami assumed Ms. BLAISE's custody and detained her at San Juan Staging Facility, in San Juan, Puerto Rico, and later transferred her to Miami Staging Facility, in Miami, FL. On February 21, 2025, ERO Miami transferred Ms. BLAISE to ERO Louisiana's (LA) custody, and ERO Louisiana detained Ms. BLAISE at Richwood Correctional Center (RCC), in Monroe, LA. On April 5, 2025, ERO Louisiana transferred Ms. BLAISE to BTC.

On February 21, 2025, during her intake screening, Ms. BLAISE presented with an elevated blood pressure (BP) reading of 252/148 millimeters of mercury [mmHg]; normal less than or equal to 120/80 mmHg]. A registered nurse (RN) documented her reported history of hypertension (HTN) and consulted with an on-call advanced practice provider (APP). The APP ordered clonidine (HTN medication) 0.2 milligrams (mg), one tablet, STAT (immediately), by mouth (PO), and amlodipine (HTN medication) 10 mg tablet, PO, daily, via pill line, and instructed the RN to initiate BP checks, in the evening, for five days. The RN cleared Ms. BLAISE for general population (GP).

Between February 24 and April 4, 2025, health staff at RCC managed Ms. BLAISE's HTN until her transfer to BTC on April 5, 2025.

On April 5, 2025, during her intake screening, Ms. BLAISE presented with elevated BP reading of 165/90 mmHg. A BTC RN consulted with an on-call APP, who ordered the continuation of Ms. BLAISE's medications, instructed the RN to schedule a next-day provider appointment, and cleared Ms. BLAISE for GP.

On April 25, 2025, at 8:41 p.m., custody staff reported a medical emergency in Ms. BLAISE's dormitory via hand-held radio. At 8:44 p.m., health staff arrived and found Ms. BLAISE unresponsive with no pulse. An RN performed a sternal rub and administered an ammonia inhalant (used to revive people who fainted), but Ms. BLAISE did not respond. At 8:45 p.m., health staff activated emergency medical services (EMS) via 911 and obtained Ms. BLAISE's blood sugar reading of 86 milligrams per deciliter [(mg/dl); normal range 70-90 mg/dL]. At 8:58 p.m., EMS personnel assumed Ms. BLAISE's care and noted she remained unresponsive, apneic, and pulseless, with fixed, dilated pupils. At 9:03 p.m., the EMS personnel's supervising physician instructed them to cease all life-saving measures and declared Ms. BLAISE deceased.

Mortality Finding:

As a result of Ms. BLAISE's death, the IHSC Assistant Director requested a mortality review to learn from her death by reviewing the care provided and the circumstances leading up to her death. The goal of the mortality review is to determine the appropriateness of clinical care;

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ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined BTC did not provide Ms. BLAISE medical care within the safe limits of practice. Specifically, the MRC determined, 1) BTC health staff deviated from basic life support (BLS) protocols; 2) Health staff failed to initiate cardiopulmonary resuscitation (CPR), despite Ms. BLAISE's pulselessness; and 3) Health staff did not include Ms. BLAISE's vital signs with their documentation of the emergency response.

The MRC convened on July 10, 2025, and consisted of the following IHSC members: Dr. (b)(7)(F),(b)(6) (b)(7)(C), medical doctor, clinical director, physician reviewer; Captain (b)(7)(F),(b)(6) (b)(7)(C), registered nurse, eastern regional nurse manager, nurse reviewer; Commander (CDR) (b)(7)(F),(b)(6) (b)(7)(C), physician assistant, APP reviewer.

The following summary of events was based on: 1) Ms. BLAISE's BTC medical records; 2) Ms. BLAISE's EMS report; 3) incident and notification reports; 4) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 5) Ms. BLAISE's BTC detention file; 6) video footage from April 25, 2025; 7) an on-site review and staff interviews conducted by Lieutenant Commander (LCDR) (b)(7)(F),(b)(6) (b)(7)(C), at BTC, on June 17, 2025; 8) applicable policies and procedures; and 9) applicable accreditation standards.

Summary of Events

On February 21, 2025, an RCC RN completed Ms. BLAISE's intake screening and documented her reported history of HTN. The RN noted normal vital signs, except an elevated BP reading of 252/148 mmHg, and consulted with an on-call APP. The APP ordered clonidine 0.2 mg, one tablet, STAT, PO, amlodipine 10 mg tablet, PO, daily, via pill line, instructed the RN to initiate BP checks, in the evening, for five days, and cleared Ms. BLAISE for GP.

On February 24, 2025, an APP completed Ms. BLAISE's initial physical exam and documented her reported history of HTN. The APP noted Ms. BLAISE's continued elevated BP readings, ordered routine labs [complete blood count (CBC), comprehensive metabolic panel (CMP), lipid panel, hemoglobin A1c, and thyroid function tests], a heart healthy diet, and daily BP checks, with nursing instruction to notify a higher-level provider of any BP readings over 180/110 mmHg, and cleared Ms. BLAISE for GP. Ms. BLAISE's lab results showed an elevated creatinine, low estimated glomerular filtration rate (eGFR) and blood urea nitrogen/creatinine ratio (indicative of decreased kidney functioning), a mildly elevated low-density lipoprotein ("bad" cholesterol), and otherwise normal results.

On February 24 and 27, 2025, an RN documented pill line medication refusals for Ms. BLAISE's BP medication and educated her on the risks and potential outcomes of refusing her BP medication.

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Between February 25 and March 1, 2025, health staff checked Ms. BLAISE's BP on four occasions and documented elevated findings (systolic pressures above 160 mmHg and diastolic pressures above 100 mmHg).

On March 5, 2025, an APP completed Ms. BLAISE's HTN and lab follow-up evaluation, diagnosed her with chronic kidney disease (CKD), noted her BP medication noncompliance, and her report of a left-sided, posterior headache for five days. The APP ordered follow-up labs, daily BP checks, losartan (HTN medication) 50 mg, one tablet, twice daily (BID), PO for 90 days, and continued her amlodipine. The APP educated Ms. BLAISE on the importance of diet and exercise and medication compliance, and scheduled a follow-up appointment in 30 days.

Between March 6 and 25, 2025, nursing staff documented Ms. BLAISE refused her pill line medications on nine occasions.

On April 1, 2025, an RN evaluated Ms. BLAISE for recurring swollen tonsils and itchy eyes. The RN noted no redness or throat swelling and consulted with an on-call APP. The APP ordered loratadine (allergy medication) 10 mg, one tablet, PO, daily for seven days. The RN scheduled an APP follow-up appointment and instructed Ms. BLAISE to return to the medical clinic if her condition did not improve.

- On the same date, an APP evaluated Ms. BLAISE for sore throat, documented enlarged tonsils with no exudate or abscess, and ordered hydralazine (HTN medication) 50 mg, one tablet, PO, BID for 90 days, an injection to decrease her immune response, an antibiotic, and an analgesic. The APP educated Ms. BLAISE on the importance of medication compliance and instructed her to follow up as needed.

On April 5, 2025, ERO Louisiana transferred Ms. BLAISE to BTC.

- On the same date, a BTC RN completed Ms. BLAISE's intake screening and documented normal vital signs, except an elevated BP reading of 165/90 mmHg, and her history of HTN. The RN consulted with an on-call APP, who ordered the continuation of Ms. BLAISE's medications, instructed the RN to schedule a next-day provider appointment, and cleared Ms. BLAISE for GP.

On April 7, 2025, an APP completed Ms. BLAISE's initial physical exam and documented normal vital signs, and her history of HTN medication noncompliance. The APP ordered routine labs (CBC, CMP, lipid panel), which showed an elevated eGFR and creatinine, a heart healthy diet, BP checks for seven days, increased Ms. BLAISE's hydralazine dosage from 50 to 75 mg, scheduled a follow-up preventive health assessment in five months, and cleared her for GP.

On April 15, 2025, an APP completed Ms. BLAISE's BP and lab follow-up evaluation, noted her BP medication noncompliance, and ordered a renal diet, repeat chronic care labs in 30 days,

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and hydrochlorothiazide 12.5 mg, daily, PO. The APP educated Ms. BLAISE on increasing her fluid intake, exercising daily, and avoiding salty snacks and processed foods.

On April 25, 2025, at approximately 8:41 p.m., custody staff announced a medical emergency at Ms. BLAISE's dormitory via hand-held radio.

- At approximately 8:44 p.m., responding health staff arrived at Ms. BLAISE's dormitory. An RN found Ms. BLAISE on the ground, unresponsive to verbal commands, and with no detectable pulse. The RN performed a sternal rub, with no response, and administered an ammonia inhalant; however, Ms. BLAISE remained unresponsive.
- At approximately 8:45 p.m., health staff activated EMS via 911 and noted Ms. BLAISE had a pulse and labored breathing after administration of the ammonia inhalant. During the health staff's response, a health staff member obtained Ms. BLAISE's blood sugar reading of 86 mg/dl.
- Between 8:45 p.m. and 8:58 p.m., health staff monitored Ms. BLAISE pending EMS personnel's arrival.
- At approximately 8:58 p.m., EMS personnel arrived at BTC, assumed Ms. BLAISE's care, and noted she remained unresponsive, apneic, and pulseless, with fixed, dilated pupils.
- At approximately 9:03 p.m., the EMS personnel's supervising physician pronounced Ms. BLAISE deceased.

Ms. BLAISE's final autopsy report is pending.

The MRC used the following accreditation standards for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, and National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The MRC identified the following program strengths, weaknesses, and recommendations during this review.

Strengths

The MRC identified the following strengths: 1) BTC health staff managed Ms. BLAISE's malignant HTN (severely elevated BP) appropriately; and 2) Health staff responded to the medical emergency within four minutes.

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Weaknesses

1. Inappropriate Emergency Care

- On April 25, 2025, responding health staff did not recognize the difference between normal breathing and agonal and apneic breathing, and they did not appropriately treat Ms. BLAISE's presenting symptoms during the medical emergency.
- On April 25, 2025, responding health staff did not implement BLS interventions, such as rescue breathing, pulse checks, CPR, or AED application, during Ms. BLAISE's medical emergency.
- On April 25, 2025, responding health staff did not obtain Ms. BLAISE's vital signs during the emergency.
- On April 25, 2025, health staff reported Ms. BLAISE had fluctuating oxygenation levels; however, there is no documentary evidence that health staff administered any oxygen.

Applicable standards of care for this finding:

- NCCHC: Standards for Health Services in Jails, 2018: J-B-07, *Emergency Services and Response Plan*.
- NCCHC: Standards for Health Services in Jails, 2018: J-B-08, *Patient Safety*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section I. *Emergency Plans*.
- PBNDS 2011 with 2016 Revisions: Part 1.1, Emergency Plans; section II. *Expected Outcomes*.
- NCCHC: Standards for Health Services in Jails, 2018: J-E-08, *Nursing Assessment*.
- NCCHC 2018 standards: J-E-09, *Continuity, Coordination, and Quality of Care During Incarceration*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section II. *Expected Outcomes*.
- NCCHC: Standards for Health Services in Jails, 2018: J-C-08, *Training*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- AHA: 2020 American Heart Association Guidelines for *Cardiopulmonary Resuscitation and Emergency Cardiovascular Care*. *Circulation*.

2. Documentation

- On April 25, 2025, during staff interviews, responding health staff stated they attempted to obtain Ms. BLAISE's vital signs during her medical emergency on April 25, 2025;

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however, the health staff did not document their attempts in Ms. BLAISE's medical record.

- On April 25, 2025, health staff who responded to Ms. BLAISE's medical emergency did not document a physical assessment in her medical record.

Applicable standards of care for this finding:

- NCCHC: Standards for Health Services in Jails, 2018: J-E-08, *Nursing Assessment Protocols and Procedures*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section I. *Emergency Plans*.
- PBNDS 2011 with 2016 Revisions: Part 1.1, Emergency Plans; section II. *Expected Outcomes*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance Division (HCCD).
- The IHSC DAD of HCCD will share these findings through appropriate communication channels to ICE, and BTC's facility administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCCD unit and ICE will ensure a corrective action plan, if applicable, is implemented and sustained.

End of report.